

SUMMARY RECORD OF THE THIRD MEETING

(WHO Conference Hall, Manila)
Tuesday, 23 September 2008, at 14:00

CHAIRPERSON: Dr Francisco Duque III (Philippines)

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1. CONSIDERATION OF DRAFT RESOLUTION

The Committee considered the following draft resolution:

1.1 Expression of appreciation to Dr Shigeru Omi (Document WPR/RC59/Conf. Paper No.1)

Decision: The draft resolution was adopted (see resolution WPR/RC59.R2).

2. PROGRAMME BUDGET 2006–2007: BUDGET PERFORMANCE (FINAL REPORT): Item 9 of the Agenda (Document WPR/RC59/3)

The DIRECTOR, PROGRAMME MANAGEMENT, introducing agenda item 9, explained that the regular budget that had been approved by the World Health Assembly in 2005 had amounted to US\$ 76 505 000, a 6.2% increase from the 2004–2005 approved programme budget of US\$ 72 036 000. The revised working allocation, as at 31 December 2007, had been US\$ 74 748 000.

Annex 1 showed the changes in the overall amount of the original budget since its presentation to the Regional Committee in September 2004, up to 31 December 2007. Annex 2 showed the detailed revised working allocations, and the final financial implementation for all areas of work.

The obligations incurred, as at 31 December 2007, for the regular budget showed a total of US\$ 74 748 000, or 100% of the revised working allocation. Activities financed from other sources of funds had amounted to US\$ 101 425 481. Columns 9 and 10 showed the total implementation of all funds and implementation by all areas of work as a percentage of all funds implemented. Columns 12 and 13 showed the change by area of work and an overall increase of US\$ 4 028 029 or 2.34% over 2004–2005.

Dr Toshiyasu SHIMIZU (Japan) commended the Regional Office on the successful implementation of the programme budget, with 100% utilization of the operating budget; the achievement of expected results had been measured by appropriate indicators, thereby providing transparent accountability.

Dr DING Baoguo (China) was pleased that the implementation rate for the regular budget was 100%, the result of a joint effort by the Regional Office and the offices in Member States; however, there was considerable variation in some areas, with disappointing implementation that should be improved upon. He hoped that the Regional Office would address that issue in order to help Member States achieve the Millennium Development Goals and to focus attention on groups that were vulnerable and the subject of discrimination. The overall figure of US\$ 102 000 000 for voluntary contributions seemed relatively low when compared with other WHO regions, particularly in light of the larger population and lesser degree of development of the Western Pacific Region. Similarly, the 64% implementation rate for voluntary contributions was not high, and he believed that the Regional Office needed to improve that in order to ensure that more funding would be forthcoming. Because many voluntary contributions were earmarked, some activities were underfinanced despite the total amount of funding, and that was common to all regions. The establishment by WHO in 2006–2007 of a corporate account mechanism provided more flexibility, and a dedicated core voluntary contribution fund had been set up to ensure that important activities received funding. He asked whether that initiative had helped to balance funding gaps and what steps the Regional Office had taken to ensure availability of funding through that mechanism.

In recent years, WHO had reformed its financial policies for project management, a reform that China supported. However, he hoped that the Organization would help Member States to adopt the new policies and improve implementation within their specific national contexts.

He asked for clarification on how allocations for projects that could not be completed by their stated deadlines were reallocated to other areas, and whether Member States were consulted. He also asked how the Regional Office intended to support Member States in the South Pacific in the areas of essential medicines, maternal and child health, and health financing.

It was important to look beyond the figures in the report and to consider that a successful outcome might come in under budget allocation, thereby indicating effective implementation by the Organization, while an implementation rate of 100% might not achieve the expected result. He hoped that future reports would include indications of that nature.

WHO had 36 areas of work, only 28 of which were covered in the report; the missing eight areas needed to be included for review by the Regional Committee. He also pointed out that in the most recent biennium the Regional Office had developed a number of guidelines, strategies and action plans that had not been translated into Chinese, which severely hampered the ability of China to implement and achieve WHO activities and goals.

Ms Ann BLACKWOOD (United States of America) referred to the variations in expenditures for certain activities that were supported by other sources depending on allocation, particularly the 302% increase relating to knowledge management and information technology, for which she requested clarification from the Secretariat.

Dr CHOI Chong Hee (Republic of Korea) noted that the final report was a significant improvement on the interim report submitted to the fifty-eighth session of the Regional Committee and she was pleased to see that progress had been made in flexibility of budget execution, as requested by the Government of the Republic of Korea the previous year. She reiterated that her country would like to have even better information, particularly statistical information, on how well programmes were doing to relate programme achievement to budget implementation. She noted that the slow progress in gender-equality-related programmes had been attributed to a shortage in, or late arrival of, funds. If budget allocations according to priority areas deprived other areas of funding, the Secretariat needed to set more realistic and achievable goals for those other areas.

The DIRECTOR, PROGRAMME MANAGEMENT, thanked representatives for their comments. Responding to the representative of China, he considered the overall figure of voluntary contributions to be close to the average in terms of distribution of the regular budget and that allocation across the Organization had been fair. With regard to the 64% implementation rate of voluntary contributions, the periodic allocation of voluntary contributions throughout the biennium and the need to have funds in hand to cover activities in the subsequent biennium had led to a carry-over of 36% of the total voluntary contributions received for the Region. The Organization considered 25%–35% to be an acceptable range for a carry-over percentage. While 36% was at the upper limit, it was still acceptable. Reallocation of funds that could not be used on certain programmes for the biennium in question took place only after regular monitoring and review and in consultation with country offices, first to other priority programmes within the country and then to

programmes within the Region. A post in maternal and child health was being established in Suva, Fiji, and that would help strengthen support in that area to the South Pacific countries. He noted the comments of the representative of China with regard to the translation of additional documents into Chinese.

Responding to the representative of the United States of America with regard to the figures for knowledge management and information technology, he said that, compared with the actual amount of both regular and voluntary contributions budgeted, the Regional Office had essentially kept to the budgeted amount, with the increase compared to 2004–2005 going principally towards providing IT equipment and improving connectivity with country offices, with additional benefits to technical programmes, such as communicating with country offices on the International Health Regulations (2005), country reporting under the International Health Regulations (2005), and pandemic influenza preparedness.

Responding to the representative of the Republic of Korea, he noted that all comments by Member States would be taken into account when preparing future financial reports.

3. PROPOSED PROGRAMME BUDGET 2010–2011: Item 10 of the Agenda
(Documents WPR/RC59/4 and WPR/RC59/4 Corr. 1)

The REGIONAL DIRECTOR reminded the Regional Committee that, during its fifty-seventh session in 2006, he had presented a six-year Medium-term Strategic Plan encompassing three biennial budget periods, starting with the 2008–2009 biennium, which formed the framework for WHO's results-based management within the overall context of the General Programme of Work. The draft Proposed Programme Budget 2010–2011 would be the second biennial budget under the six-year proposed Medium-term Strategic Plan, and the sixth successive biennial budget to follow the Organization-wide, results-based approach to programme management.

In focusing the work of the Organization around 13 cross-cutting strategic objectives over a six-year time-frame, the Medium-term Strategic Plan reflected the cross-cutting nature of WHO's work and allowed a strategic and longer-term approach to collaborative programmes.

Since the Medium-term Strategic Plan laid out WHO's strategic direction for 2008–2013, the Organization-wide and regional expected results for 2010–2011 remained largely the same as those for the biennium 2008–2009. However, the draft Proposed Programme Budget 2010–2011 included some shifts in emphasis, reflecting the evolving global and regional health situation and the corresponding changes needed in WHO's work. For example, there was a new expected result on climate change and its impact on global health in strategic objective 8, in line with the need for WHO to expand its work in that increasingly important area. Another new expected result, found in strategic objective 10, concerned patient safety, an issue that had been discussed at meetings of WHO's governing bodies and was recognized as an area needing greater attention in all parts of the world.

The draft Proposed Programme Budget 2010–2011 of the Western Pacific Region provided the operational direction for WHO in the Region for the two-year period beginning in 2010.

The Proposed Programme Budget 2010–2011 of the Western Pacific Region would be presented during the sixtieth session of the WHO Regional Committee for the Western Pacific in 2009. The content and format of the Proposed Programme Budget 2010–2011 of the Western Pacific Region would be similar to

the Programme Budget 2008–2009, ensuring continuity and comparability across bienniums. It would be developed in close collaboration with WHO country offices, the Regional Office and WHO Headquarters, and would be aligned with the Organization-wide Programme Budget 2010–2011.

WHO received its funding principally through assessed contributions from Member States and voluntary contributions. Assessed contributions were gradually accounting for a smaller proportion of the total resources received, and reliance on voluntary contributions provided by a limited number of partners was increasing. Financing the programme budget required efficient management of the different sources of income; a key challenge was ensuring alignment between the activities planned and the resources mobilized.

A significant constraint during the 2006–2007 biennium had concerned the specificity of much of the funding provided to the Region, with consequent underfunding of certain activities despite overall robust funding. Although income had grown substantially in 2006–2007 relative to 2004–2005, it had not been an even growth across all areas of work or countries and areas. As a result, there had been incomplete alignment of available resources with the programme budget, with consequent underfunding of certain activities.

Through donor dialogue, the Organization hoped to increase the proportion of less tightly earmarked funds in coming bienniums to ensure financing of priority funding gaps in the programme budget and to improve the performance of WHO in the Region.

The DIRECTOR, PLANNING, RESOURCE COORDINATION AND PERFORMANCE MONITORING, presented the Amended Medium-term Strategic Plan 2008–2013 and draft Proposed Programme Budget 2010–2011 as a continuation of the Programme Budget 2008–2009 that aimed to provide greater clarity in line with the changing income patterns and changing global environment in which WHO had to carry out its work (Annex 1).

Dr Airambiata METAI (Kiribati) thanked the Secretariat for the documents under discussion, in which funding gaps had been identified. He said country-specific gaps should still be addressed, however, if the Millennium Development Goals were to be reached. He asked whether the WHO country budget could be used on pressing needs, such as small-scale infrastructure to improve central-level communication with health posts, or for long-term training of doctors and specialists. WHO had provided Kiribati with computers for central health statistics, but it seemed that was to end. He wished to know how the Organization could best support his country.

Dr VILLAVERDE (Philippines) said his country supported the organization-wide, results-based approach of the draft budget document and the fine-tuned regional indicators and realistic targets to guide Member States on the priority areas for the biennial budget. The three-segment presentation was also appreciated. He advocated allocation of a larger share of the regional budget to country budgets, in the interests of better planning at the country level and better alignment with the specific needs of countries' medium-term plans. That would strengthen the sector-wide approach to health and reduce the spending of regional funds on activities outside country plans.

Ms BLACKWOOD (United States of America) appreciated the first opportunity to discuss the draft Proposed Programme Budget 2010–2011, which indicated certain shifts between strategic objectives; she asked that more detail on those shifts be provided for discussion at the Executive Board. She called also for more information on the new category for activities related to partnerships, and was of the opinion that

the Organization had signed up to too many too quickly. While the United States valued many of the WHO partnerships, WHO was not in a continuing position to host or associate with every new health partnership that approached the Organization. She asked that the information on tracking and monitoring received by the Governing Bodies be consistent with the activities category, and wondered how the partnership activities outside the budget envelope would be documented. In the category on outbreaks and crises, while difficult to predict, a reasonable baseline ought to be included in the programme budget, with a plan on how to move forward regionally and at Headquarters; a separate emergency fund could be established, drawing on the experience of other organizations. Such a reserve fund would ensure that resources were available to respond rapidly in the event of a crisis and replenished with donations after the event. Moving these activities to a separate category may augment transparency, particularly when unexpected events resulted in different resource impacts than those anticipated.

The United States supported steps to categorize voluntary contributions in terms of flexibility, and to make a core account of voluntary contributions. To address the funding gaps for strategic objectives 12 and 13, she agreed that levying a charge on the budgets of technical activities would make for more stable resources, but the proposal to raise the regular budget funding for those strategic objectives would result in disproportionate reliance on regular funds to subsidize the administration of voluntarily funded programmes and partnerships. With the continuing growth of voluntary funds, that could not be sustained, since it could threaten core WHO activities that relied on regular funding. A cost-recovery mechanism was needed that took actual overhead costs into account. She enquired how such practices were to be aligned with the high-level consultative mechanism to harmonize cost-recovery policies across international organizations.

On exchange rate adjustments, she questioned whether simply using a June-2008-based rate was a sound basis. She asked the Secretariat to provide information on other methodologies for projecting exchange rates for the Proposed Programme Budget for 2010–2011, such as weighted averages of long- and near-term rate changes. She also questioned the proposed change in approach to miscellaneous income, on which provisions for dealing with under- and over-estimation existed in the financial regulations. With the proposed change, the Organization was asking for a greater increase in assessed contributions than was necessary, since some miscellaneous income would be generated during the biennium. Handling such income as a separate appropriation would require a separate decision from the Governing Bodies on how to use those funds.

More information was needed in the 2010–2011 budget document regarding the Global Management System, particularly on budget savings in the long run, and perhaps in the coming biennium. The increase requested for assessed contributions was substantial, especially following the increase in the current biennium. While the activities were important, budgetary restraint was needed, including greater effort to identify potential offsets for absorbing proposed programme increases.

Ms ROCHE (New Zealand) thanked those who had contributed to the development of the draft Proposed Programme Budget 2010–2011, and welcomed the consultative process involved. She noted with concern, however, the modest funding for health systems strengthening and prevention and control of noncommunicable diseases; the level of funding would hamper sustainable actions in those areas. She supported the grouping of strategic objectives in the Medium-Term Strategic Plan. The work proposed under strategic objective 10 was welcome, as was the emphasis on climate change, and the comment from the United States of America on charging appropriate overheads for donations. She highlighted that if targets were achieved without

spending the full budget allocated, the remaining funds could be reallocated to other objectives. A significant proportion of funding was both extrabudgetary and targeted, and therefore hard to manage. She therefore asked that more Member States, and WHO itself, continue to encourage flexible funding.

Ms Palanitina TOELUPE (Samoa) noted that the draft Proposed Programme Budget had a more flexible programme structure, and the close linkage between the strategic objectives and the health priorities of the Region was appreciated.

She welcomed the emphasis on policy and technical support to enhance country capacity for surveillance and monitoring of communicable diseases. She also appreciated the support from the Regional Office to enable countries to implement the International Health Regulations (2005).

Her country noted particularly strategic objective 6, with its focus on health promotion and the prevention of risk factors. Samoa had recently passed its Tobacco Control Act, and needed support to tackle the industry and to control noncommunicable diseases. Strategic objective 7 was also important for policies and programmes that integrated action on human rights and to improve the health status of women and the poor. Intensified primary prevention was needed in all sectors to tackle the root causes of environmental threats to health, which resulted in phenomena such as the recent dengue epidemic in the Pacific islands. Nutrition, food safety and food security were equally important; and work on improving governance and health systems should remain on the Region's agenda. She asked how the partnership and collaboration arrangements, with their different dynamics, would fit in with the Paris Declaration on harmonization of donor assistance that was observed by WHO. Finally, she drew attention to implementation of the current financial system: she held that the Global Management System had not been properly explained or facilitated, which threatened delays in implementation of activities.

Dr LUVENI (Fiji) said her country welcomed the format of the budget, with tabulation of all sources of funding and distribution across strategic areas. The contribution of various parties was appreciated, with voluntary contributions that complemented other sources. She praised the new expected results on climate change in strategic objectives 8 and 10. While acknowledging the need for changes in presentation, she asked for the present format to be maintained until the end of the current strategic plan, for ease of understanding. She called for greater funding for eye care, especially trachoma, but also other causes of preventable blindness, given the commitment to the global goal of eliminating avoidable blindness by 2020. That would require the allocation of financial and human resources by WHO.

Ms Jan BENNET (Australia) welcomed the modest increase in total funding for 2010–2011, noting that the regional allocation of estimated voluntary contributions had increased by 0.2% for that period, and that the proportion of the regional budget from such contributions was high, with a possible impact on flexibility. Several indicators of the Medium-Term Strategic Plan had been revised or deleted, for better alignment with the Draft Proposed Programme Budget. However, there should be clearer identification of any gaps in funding, since that would help development partners decide where to allocate resources.

She voiced her country's concern about the uneven distribution of funds between communicable diseases on the one hand and noncommunicable diseases, violence and injury prevention on the other: 40% compared with 4.24%. It might be appropriate to reallocate some resources. Similarly, strategic objectives 10 and 11 were allocated only 14.5% of the regional budget, even although health systems strengthening was a

critical area of work. In an increasingly decentralized environment, it would be good if levels of funding for country, intercountry and regional activities could be presented more clearly.

Ms Joanna TEO (Singapore) applauded the continuing healthy overall budget for the Region, noting the increase in funding for management of water and sanitation, and control of vectorborne diseases. That reflected the shift of emphasis needed to address climate change issues and stop public health problems at the source. Continued prioritization and funding of communicable disease control supported programmes for the prevention and treatment of tropical and vaccine-preventable diseases. Equitable access to essential health care, clean water and sanitation would further help vulnerable populations. The Western Pacific Region was still under threat of an avian influenza pandemic, which showed the importance of WHO's emphasis on increasing outbreak alert and response capabilities. She hoped that WHO would continue to fund the improvement of laboratory and research capacity, especially for avian influenza.

Dr SHIMIZU (Japan) focused on the draft Proposed Programme Budget 2010–2011, which was part of the Medium-Term Strategic Plan that had begun in the current biennium. That implied that there should be no major changes in the next biennium; he enquired whether changes of any kind had been made. He understood that the Organization was appealing to donors and partners to provide core voluntary contributions, and asked how such contributions were being utilized, and how successful the Organization had been in raising them.

The presentation by the Director, Planning, Resource Coordination and Performance Monitoring from WHO Headquarters had shown the WHO budget to be unchanged from 2008–2009, although a small increase was proposed. He asked for an exact figure.

With regard to assessed contributions, he called for zero nominal growth and a major effort to reduce the core budget. For strategic objectives 12 and 13, he did not regard it as reasonable to cover the financial gap of programme support costs by using 60% of assessed contributions and 2.5% of common administration costs.

Dr MALAU (Papua New Guinea) commended the Secretariat on the preparation of the draft Proposed Programme Budget 2010–2011 and the analysis provided to the Regional Committee. He expressed appreciation to partners that had made voluntary contributions, which facilitated the work of WHO in the Region. Noting the evolving complexity of global health financing mechanisms and the conditions at the country level recommended under the 2005 Paris Declaration on Aid Effectiveness, he supported the call made by the representative of Samoa for guidance on how the Regional Office would support countries in developing their sectorwide approaches and, in particular, in ensuring the effective management of resources for health programmes.

Dr CHOI (Republic of Korea) endorsed the efforts to separate partnership and collaborative arrangements in the draft Proposed Programme Budget, which reflected moves towards greater efficiency and transparency in budget management. However, the segment related to outbreak and crisis response appeared to have been included under partnerships, and further information was requested regarding the proposal to estimate the budget for that segment closer in time to the implementation period. Member States understood that because needs in that area could not be predicted, budget revision might become necessary, but budget planning should not be delayed merely to avoid the administrative complexity of such revision. Although it might not be possible to allocate a precise amount, the basic direction should be set out.

Table 2 in the introduction in the draft Proposed Programme Budget document indicated substantial reductions in the allocations for strategic objectives 5 and 9 compared with those for 2008–2009. However, there was no explanation for those reductions, only reasons for the increases in allocations for other objectives. Member States should be provided with adequate explanations for all adjustments in order to understand why they had been made.

Dr DING (China) said that, in preparing country collaboration budgets, more consideration should be given to the specific conditions of each country, and that the country cooperation strategies should be fully used. His country supported zero nominal growth for the Proposed Programme Budget 2010–2011. He had noted, however, that other Member States had favoured an increase in the budget and asked whether the Secretariat had a good solution.

Dr BANATVALA (United Kingdom of Great Britain and Northern Ireland), commending much in the draft Proposed Programme Budget, said that the discussion offered an opportunity to assess topics covered in other agenda items against the budget proposals. There seemed to be a significant gap between the needs identified in the document on noncommunicable diseases (document WPR/RC59/6) and the funding available. He accepted that priorities had to be set and micromanagement by Member States avoided, but the morbidity and mortality due to noncommunicable diseases in the Region were such as to warrant their being given the highest priority. He supported those Member States who had called for further consideration of whether sufficient funding was being directed towards countering cardiovascular disease, cancer, mental illness, diabetes and chronic pulmonary disease.

The REGIONAL DIRECTOR said that the paucity of information about the response to the specific needs of countries was due to timing: traditionally, dialogue with Member States on the detailed operational plan of that response began in the November of the budget cycle. Obviously he wanted to allocate as much funding as possible to priorities, but he had to listen to the countries' needs and then perform a balancing act. That process would thus begin, through the country offices and the WHO Representatives, in a couple of months' time.

In response to the requests for help with infrastructure or supplies and equipment, which he interpreted to mean computers, he reminded delegates that, although WHO should provide minimal essential supplies and equipment, it was a technical agency. Sectorwide approaches provided a means for eliciting donor funding for physical facilities.

Concerning the balance of funding for noncommunicable diseases, he recalled that strategic objectives 3 and 6 were linked. Moreover, much groundwork had been laid in many Member States in the Region, for instance the WHO STEPwise approach to surveillance was being introduced in many countries. In the Secretariat's experience, the combined budget for those two strategic objectives, US\$ 42 million—not much higher than in the previous programme budget, was thought to be reasonably sufficient. He would, however, try to mobilize more resources.

He would have liked to increase funding for emergencies, disasters and crises, but more funding was needed for other priority areas. Nevertheless, he assured the Committee that, in the event of a crisis, the Organization would work hard to mobilize funds, as it had done, for example, following the tsunami in the Region in 2004.

Everybody seemed to agree that health systems strengthening, covered by strategic objectives 10 and 11, was crucial. Funding allocated to those two objectives was greater than in the previous biennium, but in finalizing the budget he would take Member States' views into account and make any necessary changes.

Concern had been expressed that the introduction of the Global Management System had delayed implementation of some activities. The initiative had been launched and the challenge was to overcome the small hitches that had been experienced. Staff across the Organization were doing their utmost to solve problems, and he pleaded with Member States to bear with the Secretariat in the meantime.

Much of WHO's funding currently came from extrabudgetary sources and most of those funds were closely tied to specific programmes. Other important programmes, such as health system strengthening, remained underfunded. The Organization was negotiating with donor agencies for more flexibility. In the Programme Budget 2006–2007, 9.3% of the US\$ 56 million had been designated as core contributions. In the current biennium, the figure had risen to 9.9%, and he wanted a further increase in the biennium 2010–2011, so that more funding could be allocated to essential programmes that were not necessarily appealing to donors.

The DIRECTOR, PROGRAMME MANAGEMENT assured the representative of Fiji that the title of strategic objective 3 had been modified to include the words "to prevent and reduce visual impairment". Resource constraints had precluded programmes on that matter in the Region, but discussions were ongoing with the Australian Government and nongovernmental organizations, such as Vision 20/20, regarding support for projects to reduce and prevent avoidable blindness, including control of trachoma.

The questions about sectorwide approaches and aid effectiveness and WHO's roles and activities would be dealt with under the next agenda item (health system strengthening and primary health care). Part of the Secretariat's strategy was providing support to health ministries to draw up national health plans, as they were the first requirement for governments to be able to embark on a sectorwide approach. Support was being provided through technical cooperation.

The DIRECTOR OF PLANNING RESOURCE COORDINATION AND PERFORMANCE MONITORING, WHO Headquarters, assured delegates that their comments would be taken into consideration along with those from other regional committees during the finalization of the budget.

The introduction to Annex 2 of document WPR/RC59/4 indicated that the partnerships included in the draft Proposed Programme Budget were those that contributed directly to the achievement of the Organization-wide expected results for a given strategic objective. Those partnerships that were outside the budget did not contribute directly to those results even though they supported several strategic objectives (see the annex to Annex 2).

On programme support costs, she assured the representative of the United States of America that WHO participated with other United Nations organizations in harmonizing cost-recovery policies, but nevertheless the average collection rate was only about 7%, an insufficient percentage. Many measures were being taken to increase income in order to reduce the strain on the regular budget, but it was not possible to fully finance those costs.

The questions on miscellaneous income might have arisen from a misunderstanding. It would still be used for budget support, although under a different title: it would still be appropriated when approved by the World Health Assembly, as for example had happened in May 2008 when the World Health Assembly, in resolution WHA61.6, had resolved to appropriate funds from miscellaneous income in order to finance two appropriation sections of the Programme Budget 2008–2009. The matter would be explained more fully in the finalized text.

WHO received payments in numerous currencies, thereby complicating an already difficult situation due to currency fluctuations. The Secretariat's report to the Executive Board at its 124th session in January 2009 would break down the impact for the regions and provide more details.

She confirmed that an emergency fund would be set up. For strategic objective 5, a sum of US\$ 120 million had been programmed for those emergency preparedness issues that were considered to be part of the ordinary public health domain. The emergency fund should be in the order of US\$ 300 million to US\$ 400 million, of which only a small proportion would be needed initially.

It was too early to quantify savings and efficiencies from the introduction of the Global Management System, which was not yet fully operational or being used by all regions. Some savings had been made at WHO Headquarters as a result of redistribution of administrative staff members or mutually agreed separations.

The World Health Assembly, in resolution WHA60.11, had requested the Director-General to recommend revisions to the Medium-term Strategic Plan 2008–2013, as might be necessary. The changes being proposed were minor and were underlined in document WPR/RC59/4. She assumed that Japan's question about a small increase referred to assessed contributions. The Secretariat was asking for US\$ 56.6 million, a 6% increase, but she pointed out that currency fluctuations had appreciably benefited most major contributors that paid assessed contributions in local currencies, apart from the United States of America. She welcomed the support for more flexibility in funding as that was the key to a balanced budget and the only way to implement and fully fund the Medium-term Strategic Plan.

The DIRECTOR-GENERAL thanked representatives for their useful comments. Recalling that the draft Proposed Programme Budget under discussion was the first one for which she had been responsible, she said that it had been drawn up by the Global Policy Group, consisting of herself, the Deputy Director-General and the six regional directors. They had had difficulty deciding on a zero-growth budget, after three bienniums of increasing income; nevertheless, the Group had considered it essential to consolidate the gains by budgetary restraint. The increase that was being requested reflected the gains achieved on currency exchange rates. She accepted the comment of the representative of the United States of America that use of a point exchange rate rather than a weighted average was not fair, and that would be taken into consideration before presentation of the proposed budget to the Executive Board.

The Global Policy Group had also based the budget on the principles of transparency and accountability. The budget had thus been broken down into three components—WHO programmes; partnerships; and outbreak and crisis response – so that it would be clear exactly where money was coming from and how it was being spent in each sector. Although the finances of partnerships entered into the overall budget, they were not the responsibility of WHO. A strategic discussion on the nature of WHO partnerships was under way, and a document that had been revised on the basis of Member States' comments would be discussed at the meeting of the Executive Board in May 2010, until which time no new partnerships would be hosted.

She had noted the comments of representatives regarding the cost recovery principle. Although WHO had agreed to limit itself to levying 13% for programme support costs, many countries attempted to negotiate, and in fact WHO found that it managed to retain an average of 7%; she asked Member States to allow the Organization to cover its costs.

She recalled that the Global Management System had been set up in response to a request from the governing bodies, as an instrument for modernizing the Organization's finances and making them more transparent. The problems that were currently being experienced were those to be expected with any new enterprise resource planning system. Additional help was to be brought in to resolve the problems. The system would be extended to the other regional offices only when it had been stabilized. She asked for the indulgence of donors if reports were received later than usual.

There being no further comments, the CHAIRPERSON asked the Rapporteurs to draft an appropriate resolution.

4. HEALTH SYSTEMS STRENGTHENING AND PRIMARY HEALTH CARE (Document WPR/RC59/5): Item 11 of the Agenda

The DIRECTOR, HEALTH SECTOR DEVELOPMENT, introducing Agenda item 11, said that strengthening of health systems was increasingly recognized as a key element in achieving the universal goal of "health for all" and the health-related Millennium Development Goals. Weak, inefficient health systems all too often constrained the ability of countries to make more rapid progress towards their desired health outcomes. Health systems should be oriented towards primary health care in order to deliver better outcomes and greater public satisfaction at lower cost. Countries should address several issues, including quality, the private sector, definition of health service packages, laboratory services, the social determinants of health and equity, as well as gender and human rights.

In order to provide more support to Member States in strengthening their health systems, WHO had published *Everybody's Business: Strengthening Health Systems to Improve Health Outcomes*, which described six "building blocks" for health systems. The Regional Office had prepared a strategic plan for strengthening health systems in the Region, defining the directions for its support to Member States, with the vision of universal access to high-quality services. It was based on the principles and core values of primary health care, which would guide WHO's support to Member States in strengthening their health systems.

The Regional Office had convened a meeting of experts in August 2008 to discuss health systems strengthening and primary health care. The experts had recommended preparation of a regional strategy in consultation with Member States that built on a WHO Secretariat document entitled Strategic Plan for Strengthening Health Systems in the Western Pacific Region. They had encouraged the Regional Office to strengthen its analytical capacity and evidence on country-specific health systems issues and to share that information through a mechanism such as an Asia Pacific observatory on health systems and policies.

The Regional Committee was being asked to discuss and provide guidance on future directions for strengthening health systems in the Region and to renew its commitment to primary health care. In addition, the Committee was being asked to consider the recommendation of the expert group to set up an observatory and to adopt a suitable resolution.

Dr LUVENI (Fiji) thanked WHO for having simplified the six “building blocks” for health service strengthening. She noted that the strategic plan built upon current regional plans, such as those for human resources and health, improving access to essential medicines and health care financing. She thanked WHO for its assistance in developing human resources for health, which was a critical component of health systems. The Pacific Human Resources for Health Alliance and the Asia Pacific Action Alliance on Human Resources for Health would both be useful means for improving human resources for health in the Region. A recent meeting on health care financing in her country had resulted in the formulation of strategies and an action plan, and she looked forward to further collaboration with WHO in that field. Primary health care was an essential element of health systems, and her country was reorienting health service delivery on the basis of that principle, which would result in better health outcomes and greater public satisfaction at lower cost. She called on the Regional Director to align the WHO programme with the national health plans of Member States.

Dr SHIMIZU (Japan) agreed that it was useful to reaffirm and review the core values and principles of primary health care on the eve of the 30th anniversary of the Declaration of Alma-Ata. Strengthening health systems oriented towards primary health care would give people in all countries greater satisfaction and better health outcomes. He therefore welcomed the Strategic Plan for Strengthening Health Systems in the Western Pacific Region. At the G8 Summit, held in Toyako, Hokkaido, Japan, in July 2008, his country had presented a framework for action on global health, with health systems strengthening as a priority; other priorities were health financing, the health workforce and health information. A workshop on health systems strengthening, with the participation of international experts, was to be held later in the year, the results of which might be discussed at the next World Health Assembly. Much work would be required to elaborate an effective strategy for strengthening health systems and primary health care, and he looked forward to WHO's leadership in that respect.

Dr METAI (Kiribati) expressed appreciation for the information provided in the document before the Regional Committee, in particular regarding the six building blocks of health systems that required strengthening, which he endorsed. Kiribati had adopted a primary health care approach to the delivery of health systems—accepted as a cost-effective way of providing services—28 years previously. Obstacles to effective implementation had included inadequate facilities, lack of competent human resources, and lack of access to appropriate technology to improve communication between the central level and peripheral facilities. Staff had been unable to cope with the simultaneous implementation demands of so many programmes. Multisectoral involvement and community participation had improved the situation. The policy of the previous Government is not permitting voluntary work until funds were available for a job had been counterproductive, however, and had reduced the willingness to volunteer. While local training of nurses was satisfactory, funds and places for training of physicians and health professionals in the areas of radiography, pharmacy and laboratory investigation at suitable training institutions were often lacking. For example, the Fiji School of Medicine would take only four students per year. WHO should support countries in finding training opportunities in other institutions. For a country comprising islands spread over a vast area of ocean, good communication was essential. Fortunately, trials of communication technology to transmit information from the outer islands were producing promising results.

Kiribati acknowledged WHO support in many areas of health systems development. However, it would require further technical and financial support from WHO and other partners in that area and others if it was to attain the Millennium Development Goals.

Dr MALAU (Papua New Guinea), welcoming the inclusion of the item on the Agenda, said that, in the year since the appointment of the Minister of Health and himself, Papua New Guinea had acknowledged the need to tackle the country's poor health indicators and had renewed a commitment to primary health care and to service delivery to the rural majority and the urban poor, for example, with a view to reducing the high rates of maternal and infant mortality. The Department of Health had therefore formulated a corporate plan for the period 2009–2013 focusing on human resources, health facilities and equipment, information and communication technology, and evidence-based costing of the health sector budget. Activities in 2009 would concentrate on negotiations with the provinces in the development of a long-term strategic plan in line with the country's intended long-term development plan, which would be the national strategic plan to 2050. Effective health systems management would be vital in providing services across the country's diverse range of health care settings and in coping with new health threats at the local level. The previous decentralized system of governance and a dual legislative framework comprising the Hospital Management Act, which governed provision of hospital services, and the Public Health Administrative Act, which set out the administration of rural health services, had resulted in a disjointed approach and ineffective management at the local level. The recent Provincial Health Authorities Act would permit the establishment of a single health authority at the provincial level, and further reforms and restructuring would be based on sound management practices. Papua New Guinea looked forward to working with WHO and other partners to ensure effective health service delivery and improved health outcomes.

Dr VILLAYERDE (Philippines) affirmed his country's commitment to the strengthening of health systems based on the core values and principles of primary health care, and its support for the proposal to establish an Asia Pacific Observatory on Health Systems and Policies as a means of documenting and strengthening health systems practices and models in the Region. Health systems strengthening was one of his country's flagship programmes in its implementation strategy for health sector reform, which included all six of the health system building blocks identified by WHO. The Philippines had adopted a sectorwide approach to health development: the decentralized health system relied on strong local government and community and civil society participation, as well as donor support. Using indicators reflected in the Paris Declaration on Aid Effectiveness, a European Commission team had assessed the approach as a good global practice. The Philippines was also taking steps to enhance the gatekeeping functions of primary health care facilities as a way to improve health system performance, efficiency in the use of resources, and health outcomes, in line with its commitment to reach the poorer sections of the population and attain the Millennium Development Goals.

Dr RAHMAT (Malaysia) proposed that the Regional Committee should adopt an appropriate resolution on health systems strengthening and primary health care that endorsed the Strategic Plan for Strengthening Health Systems in the Western Pacific. In 2007, the Malaysian Ministry of Health, in partnership with other government organizations affecting the social determinants of health, had reviewed the country's primary health care approach against the primary health care core values of equity, universality and solidarity. The review had identified 20 areas for improvement, foremost among which was the strengthening of health systems to ensure attainment of the Millennium Development Goals and to meet the challenges posed by

epidemiological and demographic transition. The conclusions had been reiterated in the Kuala Lumpur Primary Health Care Declaration, arising from the Malaysian National Primary Health Care Conference in April 2008. The best measure of health system performance was its impact on health outcomes: Malaysia's national health policy, with its focus on primary health care since the 1980s, had resulted in significant improvements in health status. Malaysia was willing to share its experience and to learn from others. It looked forward to receiving further information on how the proposed Asia Pacific Observatory on Health Systems and Policies might support countries in analysing and strengthening health system performance.

The meeting rose at 17:20.



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*Presentation for the Regional Committees
September/October 2008*

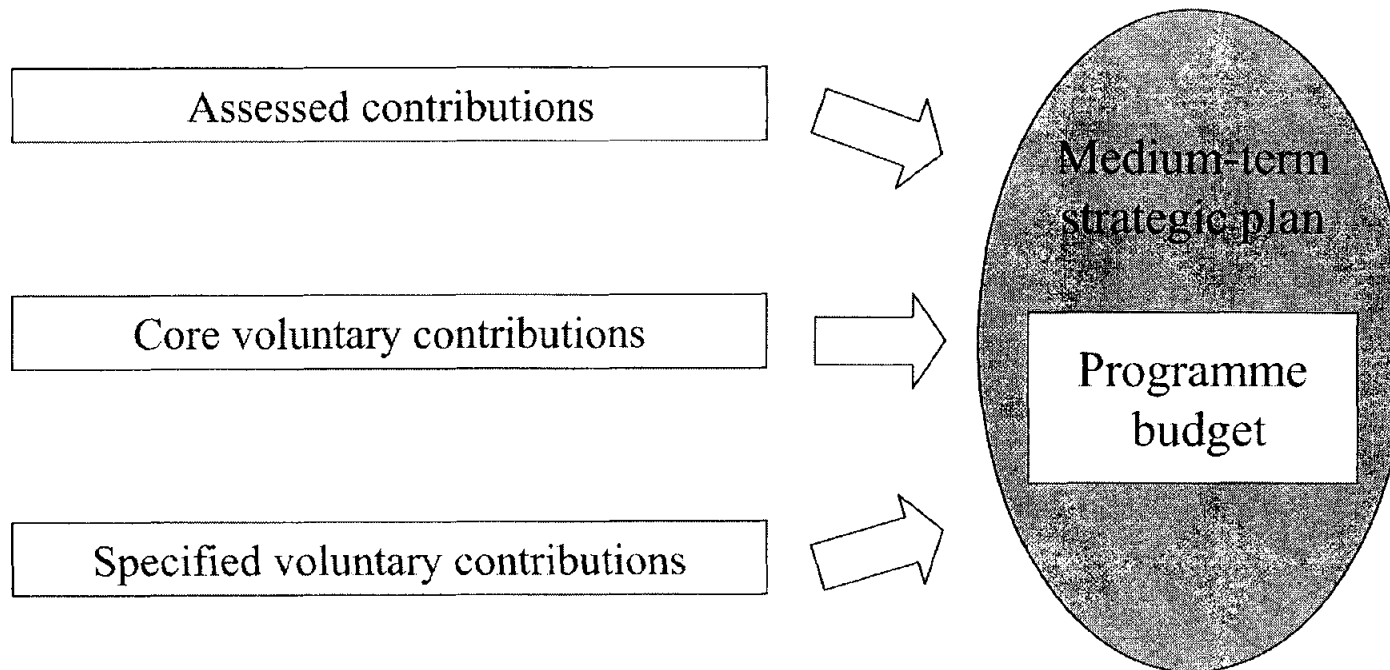
Draft amended MTSP: Strategic Objectives

- **No new** strategic objectives
- Substance of the **13 strategic objectives** unchanged
- However, some **shifts in emphasis**, reflecting the evolving global health situation and the corresponding changes needed in WHO's work



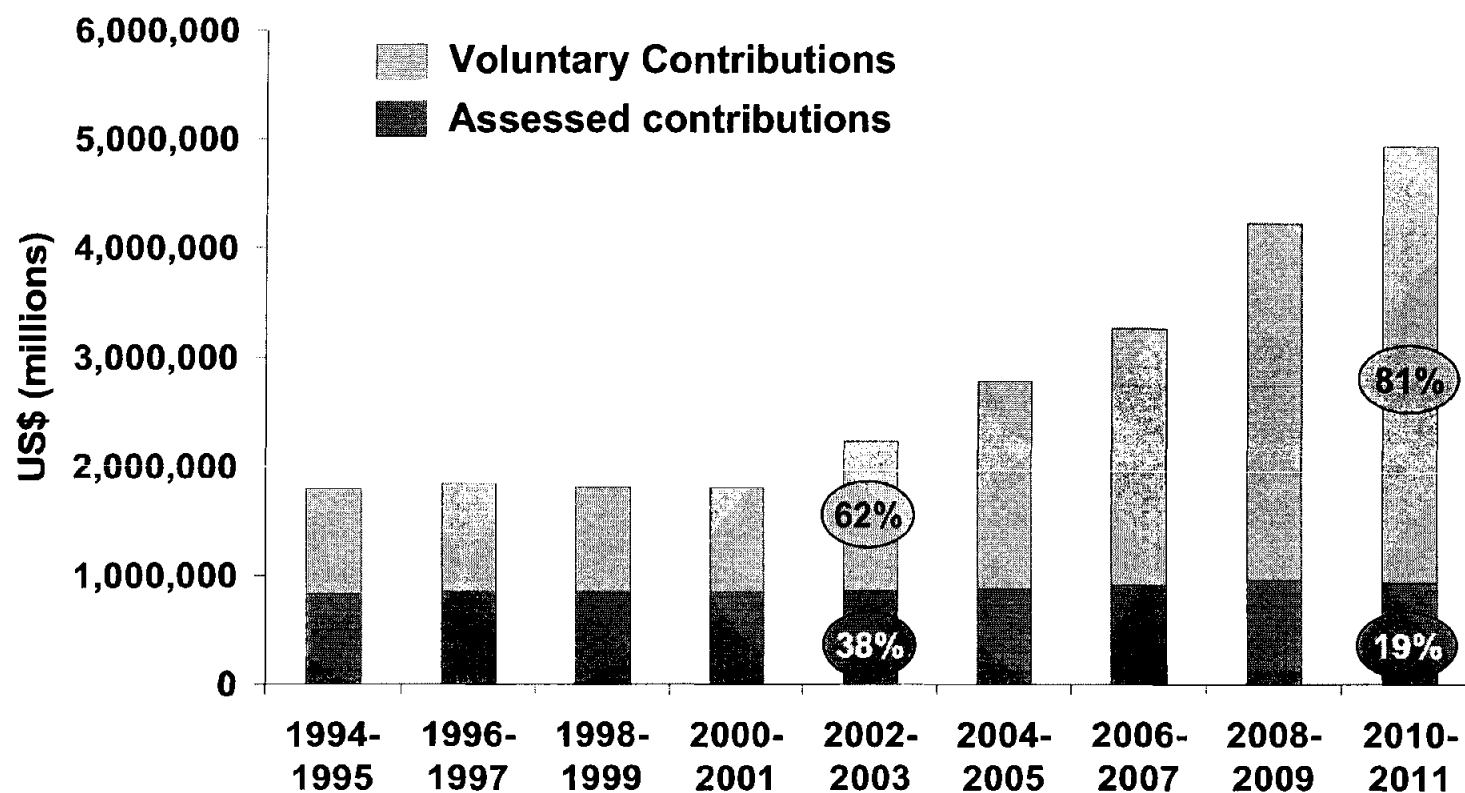
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- One Medium-term strategic plan - One Programme Budget
- Three sources of funding



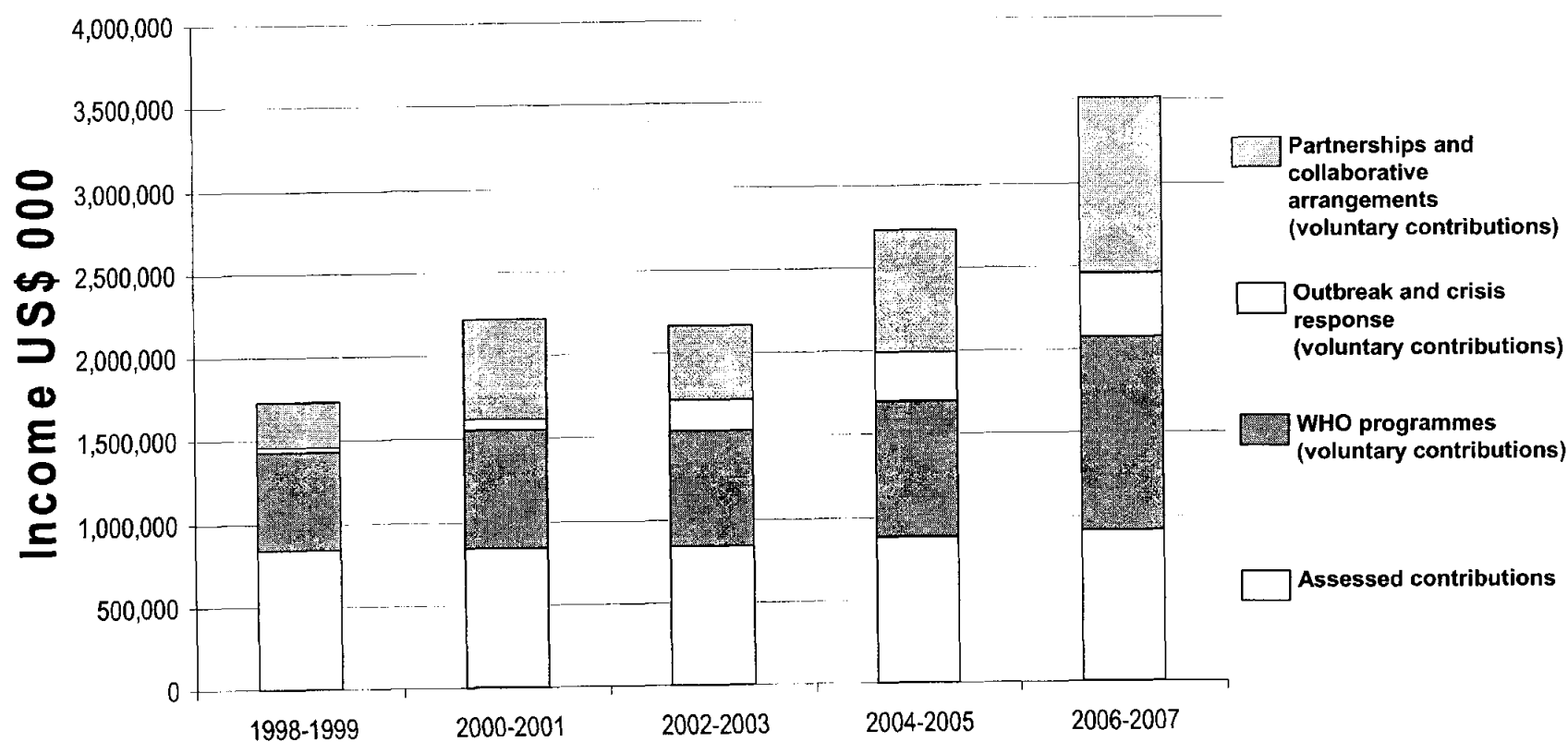
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Programme Budget growing consistently since 2002-03, with a marked increase in voluntary contributions reliance



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Development trend in the composition of WHO income



Draft Proposed programme budget 2010-2011 : challenges

- Challenge of increasing number of partnerships and collaborative arrangements:
 - many hosted by WHO and included in the programme budget, but not well delineated
 - partnership and collaborative arrangements budgets not decided by WHO solely
- Increased WHO involvement in outbreak and crisis response
- Partnerships & collaborative arrangements and outbreak and crisis response follow different dynamics and have different budget and management requirements.



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Draft Proposed programme budget, 2010-2011 in three segments

- WHO programmes
- Partnerships and collaborative arrangements
(increasing the transparency of partnerships within the
global governance of WHO)
- Outbreak and crisis response
(distinguishing crisis activities and related budget, that
are by their nature unpredictable)



Draft Proposed programme budget 2010-2011: principles (1)

- WHO needs to consolidate its growth and strengthen its implementation capacity, while ensuring, at the same time, there is a continuing focus on priorities
- Assessed contributions largely unchanged over 10 years , but need to ensure they remain at the same level in **real terms**
- Voluntary income for WHO programmes steady increase expected to continue
- Partnerships & collaborative arrangements and outbreak and crisis response are the fastest growing segments – increased importance from 2006-07 – trend expected to continue



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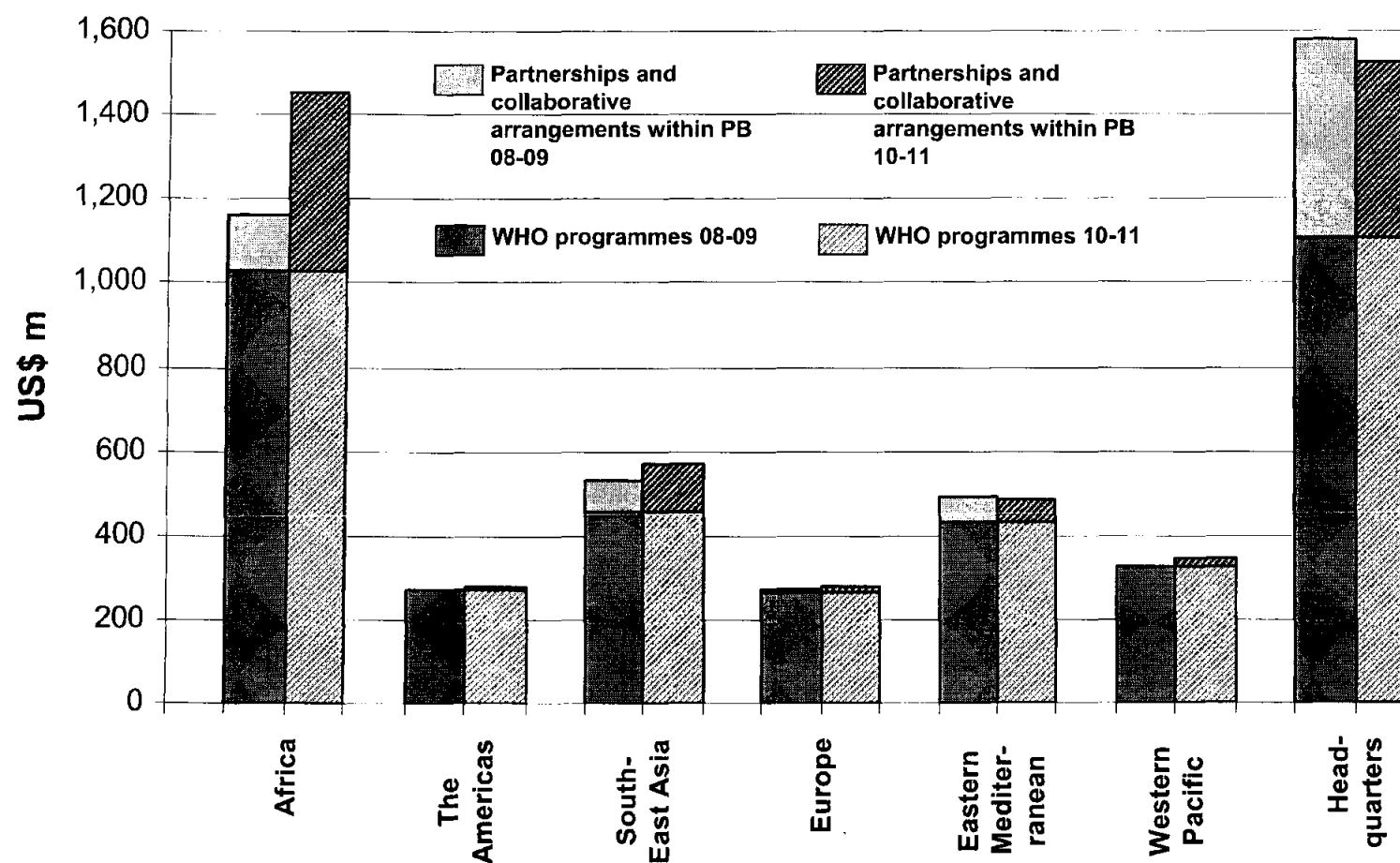
Draft Proposed programme budget 2010-2011: principles (2)

- In principle, the budget of "WHO programmes " should be unchanged from 2008-09 in nominal terms - at US dollar 3,888 million
- However a small increase is proposed to the overall budget level for "WHO programmes" due to the impact of exchange rate changes against the US dollar
- The major part of the budget will be spent in regions and countries following the principle of "70%-30%" which continues to guide the organization



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Proposed budget 2010 – 2011 compared to (revised) budget 2008 – 2009, by office: WHO programmes and Partnerships (US \$ million)



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